



PATIENT HANDOUT

IBD Fatigue: 15 Things That Can Help

Practical ideas for when you're completely exhausted and rest alone isn't fixing it

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IBD Fatigue: 15 Things That Can Help

Practical ideas for when you're completely exhausted and rest alone isn't fixing it

Fatigue in Crohn's and colitis is not just being tired. It is common, reported by around half of people with IBD even when the disease itself is in remission, and it often does not lift with sleep or rest the way ordinary tiredness does.[1][2] That makes it worth treating as its own problem, not a side effect to just push through.

START HERE

Check the Medical Causes First

Several common, treatable causes sit behind a lot of IBD fatigue. Worth raising with your GP or IBD team before assuming it is just how you feel now:

Possible cause	Why it matters
1. Iron deficiency	Common with blood loss or inflammation; a simple blood test checks it.
2. Vitamin B12, vitamin D or folate deficiency	More common in IBD, especially with small bowel involvement, and can be associated with fatigue.[4]
3. Active inflammation	Fatigue can be a symptom in its own right, even without obvious gut symptoms.
4. Thyroid function	An underactive thyroid causes fatigue and is simple to test for.
5. Medication side effects	Steroids, some biologics and other IBD medicines can affect energy and sleep.

ACTIVITY

Pace Yourself, Rather Than Push Through

It sounds backwards, but total rest is not usually the fix. For some people, carefully paced regular activity can help fatigue and prevent loss of fitness.

- 6. Break tasks into shorter chunks with a real rest in between, rather than one long push
- 7. Build in a short walk most days, even a slow one; consistency matters more than distance
- 8. Notice your own "boom and bust" pattern: overdoing it on a good day often means a worse one follows

SLEEP

Protect Your Sleep

Poor sleep and inflammation feed each other, so improving one tends to help the other.[3]

- 9. Keep a consistent bedtime and wake time, even on days you feel exhausted
- 10. If night-time bathroom trips break your sleep, talk to your team; adjusting medication timing sometimes helps
- 11. Keep screens and caffeine out of the last hour before bed

FOOD & HYDRATION

Eat and Drink Regularly

- 12. Small, regular meals help avoid the energy dips that come with skipping food
- 13. Stay ahead of dehydration, especially if diarrhoea is frequent; it makes fatigue worse on its own
- 14. If you are cutting food groups because of fatigue, get a dietitian involved rather than guessing (see the Practical Food Guide)

DON'T CARRY IT ALONE

Get the Support Around You Working

- 15. Tell one person close to you what fatigue actually stops you doing; it is easy for others to underestimate an invisible symptom

At work or in education

Fatigue is covered by the same reasonable-adjustments principles as any other IBD symptom. A later start, more frequent breaks, or the option to work from home on a bad day are all worth asking for.

DON'T WAIT ON THIS

When to Get It Checked Urgently

See your GP promptly if fatigue comes with:

Breathlessness or a racing heart at rest, dizziness or fainting, unusually pale skin, or fatigue that has come on suddenly and severely. These can point to significant anaemia, which is treatable once found.

GO FURTHER

More Support From VanceHealthHub

- **Malnutrition Calculator:** track unintentional weight change, which often runs alongside fatigue
- **The Practical Food Guide for Crohn's & Colitis:** nutrients worth checking, and when to see a dietitian
- **Health Record:** log symptoms and energy levels in one private place
- **VANCE-Ai:** a starting point for questions between appointments, not a substitute for your care team

None of these replace your GP, gastroenterologist or dietitian, and nothing on VanceHealthHub should be used to diagnose or treat a condition.

THE EVIDENCE BEHIND IT

References

Crohn's & Colitis UK · Guts UK

1. Czuber-Dochan W, et al. Review article: describing and quantifying the impact of fatigue on people living with inflammatory bowel disease, a European patients' perspective. *Aliment Pharmacol Ther*, 2020;52(1):5-13.
2. Villoria A, et al. Fatigue in out-patients with inflammatory bowel disease: prevalence and predictive factors. *PLoS One*, 2017;12(7):e0181435.
3. Ali T, et al. Sleep, immunity and inflammation in gastrointestinal disorders. *World J Gastroenterol*, 2013;19(48):9231-9239.
4. Bischoff SC, et al. ESPEN guideline on Clinical Nutrition in inflammatory bowel disease. *Clin Nutr*, 2023;42(3):352-379.

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